

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 76 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: June 10, 2021 | Provider: Karen Voss, MD — Neurology | Doc ID: RFH-2021-0610-NEURO

Neurology Follow-up

Post-TIA Secondary Prevention — Carotid Duplex Review**Subjective**

Five weeks after high-risk TIA. No recurrent neurological symptoms. Tolerating medications. BP improved at home (mid-120s/70s).

Carotid Duplex Ultrasound (05/28/2021) — Results

Left internal carotid artery: 50-69% stenosis (peak systolic velocity 210 cm/s). Right internal carotid artery: <50% stenosis. Antegrade vertebral flow bilaterally. Findings concordant with CTA.

Assessment

1. TIA, cardioembolic with contributory moderate (50-69%) left ICA stenosis — asymptomatic-range stenosis, best managed medically (no revascularization indicated for moderate stenosis).
2. Atrial fibrillation — anticoagulated (apixaban), dose appropriate for renal function (note eGFR ~38; apixaban 5 mg BID retained as only age meets reduction criterion borderline — monitored).
3. CKD stage 3b. CAD, HFrEF, T2DM — comorbid vascular risk.

Plan

- Continue apixaban 5 mg BID; intensive statin + ezetimibe (LDL goal <55); BP goal <130/80.
- Repeat carotid duplex in 6-12 months to monitor stenosis progression.
- Reinforce stroke warning signs and adherence.
- Coordinate renal dosing review with nephrology referral (declining eGFR).

Secondary Prevention Regimen

Medication	Dose	Route	Frequency	Indication
Apixaban	5 mg	PO	Twice daily	AF / cardioembolic prevention
Atorvastatin	80 mg	PO	Nightly	Atherosclerosis
Ezetimibe	10 mg	PO	Once daily	LDL lowering (added)
Sacubitril/valsartan	49/51 mg	PO	Twice daily	HFrEF / BP